



REFERRAL PACKET

When chronic symptoms don't respond to standard care, the jaw is often the missing variable.

A clinical orientation for referring physicians, dentists, and allied practitioners.

Dr. Dwight Jennings runs a tertiary referral practice in the San Francisco Bay Area focused on jaw orthopedics and the cranio-facial / systemic-symptom interface. Patients arrive with chronic headaches, fibromyalgia, sleep apnea, TMJ dysfunction, and undiagnosed pain syndromes that have not improved under mainstream care. A meaningful percentage of his patients fly in from outside California.

The framework his practice rests on is straightforward: a misaligned bite irritates the trigeminal nerve; chronic trigeminal activation produces a constellation of downstream symptoms that mainstream medicine commonly treats as separate diagnoses. Realigning the bite — a process, not a procedure, almost never surgical — quiets that irritation and, in responders, the constellation reduces or resolves.

“Curiously, I have never heard a single physician mention the idea of looking at the bite on patients having broad medical complaints.” — Dr. Jennings

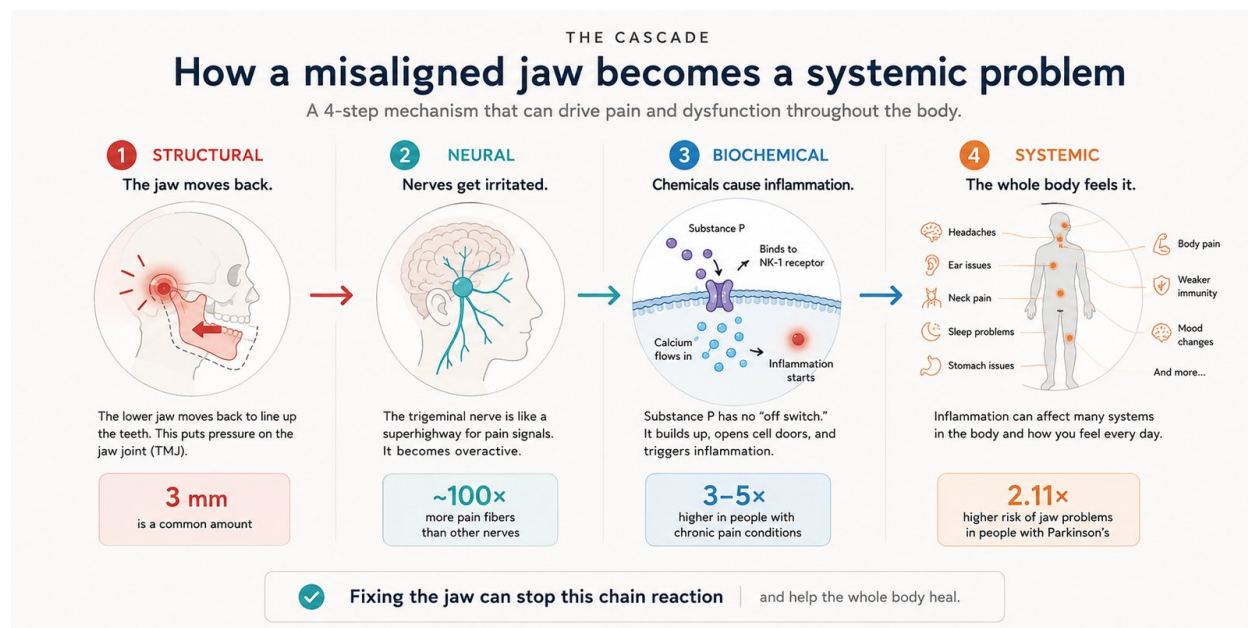
This packet contains: (1) the mechanism Dr. Jennings has been treating, diagrammed; (2) the conditions where a referral is most likely to help; and (3) how to refer a patient.



THE MECHANISM

How a misaligned jaw becomes a systemic problem.

Four steps. The first three are established neuroscience. The fourth maps to forty years of Dr. Jennings' clinical observation.



1. Structural · Jaw misalignment

A vertical-height deficiency — often inherited, often worsened by extraction-based orthodontics — forces the lower jaw to retrude to bring the teeth together. The TMJ is compressed. The chewing muscles (especially the temporalis) work continuously to hold the jaw in a non-resting position.

2. Neural · Trigeminal hyperactivation

Chronic muscle and joint irritation activates the trigeminal nerve — the body's densest concentration of nociceptive C-fibers. The nerve releases **substance P**, a neuropeptide with no reuptake mechanism. It accumulates and lingers.

3. Biochemical · Substance P at the cell

Substance P binds the NK-1 receptor, opens cell membranes, lowers cell voltage, and allows calcium influx. It is established as a primary driver of inflammatory processes in the respiratory, gastrointestinal, and musculoskeletal systems.



4. Systemic · Downstream dysfunction

Sustained substance P elevation correlates clinically with the conditions Dr. Jennings sees most often: chronic headaches, fibromyalgia, IBS, autoimmune disorders, sleep apnea, movement disorders, mood and ADD/ADHD presentations, and a broader pattern of complex chronic illness.

The treatment is undramatic. Almost never surgery. A precision-fitted removable dental appliance repositions the mandible to a physiologically optimal rest position. Substance P production downregulates. In responders, the downstream symptoms reduce or resolve.



WHO TO REFER

Conditions where a referral is most likely to help.

Dr. Jennings is not a general dentist. He is a tertiary specialist for patients who have plateaued under conventional care. The list below describes the patterns where forty years of his clinical experience suggests the bite is worth examining.

TMJ pain & dysfunction

Jaw pain, clicking, locking, ear pressure, facial pain, post-surgical TMJ complications. The original specialty of the practice — and where most patients begin.

Chronic headaches & migraines

Daily or near-daily headaches; classical migraine; tension-type or cluster patterns that have not improved on standard pharmacological care. Bite alignment is reported in the published literature to be ~85% effective at reversing chronic headaches across types.

Sleep apnea (and CPAP-intolerant patients)

Particularly indicated when CPAP has failed or been declined; when snoring, retrognathia, and chronic fatigue co-occur; or when sleep apnea presents alongside the broader chronic-illness pattern.

Movement disorders

Parkinson's, Tourette's, dystonia, torticollis, gait disorders. Not all cases respond — but a subset have a strong cranio-mandibular component and improve with alignment therapy when other approaches have not.

Surgical alternative

Patients recommended for TMJ joint replacement, condylectomy, or jaw-advancement surgery deserve a non-surgical second opinion before proceeding. Many can be treated non-surgically with precision jaw orthopedics.

Complex & undiagnosed chronic illness

Fibromyalgia, IBS, autoimmune patterns, chronic fatigue, ADD/ADHD, anxiety and depression that don't fit cleanly into a single specialty's workup. Often these patients have seen many physicians and run a long differential.



HOW TO REFER

Sending a patient to Dr. Jennings.

There is no formal referral form. A short note from you, plus the patient calling to schedule, is enough.

What to send

If you have any of the following, they help us assess fit before the first visit: panoramic dental imaging, recent MRI or CT of head and neck, sleep study, current medication list, and a brief narrative of what has been tried.

Who is a good fit

Patients who have plateaued under conventional care and where the presentation includes any one of: TMJ pain or dysfunction; chronic headache or migraine; a bite that has shifted with age or after orthodontics; chronic-illness pattern with a strong inflammatory or neuropeptide-mediated footprint (fibromyalgia, autoimmune, mood, ADD, post-viral); sleep apnea with retrognathia or CPAP intolerance; or any movement disorder where a trigeminal-mandibular contribution has not yet been ruled out.

Who is not a good fit

Patients seeking cosmetic dentistry, routine restorative work, orthodontics for adolescents without complicating symptoms, or acute dental emergencies. Dr. Jennings does not provide general dentistry.

Logistics

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This packet is informational. Nothing here constitutes diagnosis or treatment advice. Any clinical decision remains with the patient and their care team.